Advanced ALK-positive non-small-cell lung cancer (NSCLC) after crizotinib Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: National Cancer Control Programme (NCCP) 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

NCCP 00340 · VERSION 4

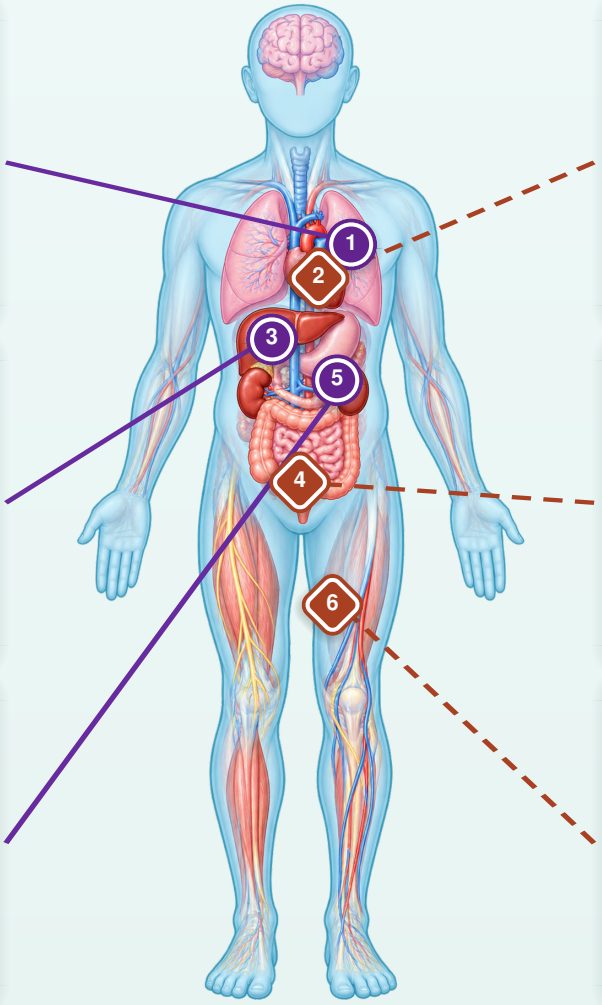
Ceritinib monotherapy

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 New cough or breathlessness

ceritinib

Possible lung inflammation needs urgent assessment.

2 Fainting or palpitations

ceritinib

Slow pulse and rhythm changes can be serious.

3 Yellow skin or dark urine

ceritinib

Blood tests detect liver changes; symptoms need urgent advice.

4 Diarrhoea or vomiting

ceritinib

Persistent symptoms can cause dehydration.

5 Persistent abdominal pain

ceritinib

Pancreatic inflammation needs prompt assessment.

6 Thirst and frequent urination

ceritinib

High glucose may need urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology urgently for new or worsening cough or breathlessness, fainting, palpitations, severe dizziness, yellow skin or eyes, dark urine, severe persistent abdominal pain, persistent vomiting or diarrhoea, poor drinking or little urine, or marked thirst with frequent urination and feeling unwell. Severe breathing difficulty, collapse or severe chest pain needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Take at the same time daily with food. Swallow capsules whole with water; do not chew or crush. Take a missed dose unless the next is due within 12 hours; ask your pharmacist if timing is unclear. Never double a dose. If you vomit, do not replace the dose; take the next at its usual time. If illness prevents you eating with the dose, contact the team. Only the prescriber can arrange a different fasting dose. Do not alternate between fed and fasting schedules or change the dose yourself.

Checks and follow-up

Before starting: ALK test, blood count, kidney and liver tests, pancreatic enzymes, fasting glucose, electrocardiogram (ECG) and symptom/medicine review. Blood count, kidneys and fasting glucose every 4 weeks; liver tests every 2 weeks for the first 3 months, then monthly. ECG after 1 month, then as needed. Extra heart, electrolyte, glucose or pancreatic tests may be needed for risks, symptoms or abnormal results; scans follow the consultant plan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Prevent and treat nausea

Ceritinib has moderate-to-high oral sickness risk. Ask for daily prevention and rescue instructions. Your team must check heart-rhythm and medicine interactions when choosing anti-sickness treatment; do not add medicines or a steroid course yourself.

Food and daily doses

If illness prevents you eating with the dose, contact the team. Only the prescriber can arrange a different fasting dose. Do not alternate between fed and fasting schedules or change the dose yourself.

Check interactions

Avoid grapefruit or grapefruit juice and St John’s wort. Check prescription, pharmacy and herbal medicines before use, including heart medicines, antibiotics, antifungals, seizure treatments and acid-reducing medicines. Do not stop a prescribed medicine without advice.

Pregnancy, fertility and feeding

Discuss pregnancy, fertility and breastfeeding before treatment. Women who could become pregnant need highly effective contraception during treatment and for 3 months afterwards; hormonal contraception may be less effective, so agree a suitable method. Do not breastfeed during this treatment. Fertility effects are uncertain; ask about preservation if relevant.

Diarrhoea, fluids and daily life

Use the agreed diarrhoea treatment promptly. Report persistent symptoms, poor intake or dehydration. Ask for help with appetite, fatigue and practical support; avoid driving or machinery if fatigue or visual symptoms affect safety.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.



Your complete treatment reference

https://sactcheck.com/patient/00340/

Public information only. No personal information is encoded.